

Patient ID: JonesPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 704/745h (94%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Last 24 Hours

Last 24 Hours: within normal range

Vital Stat	Avg	Min	Max
Heart Rate	55 bpm	39 bpm	74 bpm
Breathing	17 brpm	12 brpm	27 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

Patient clinical status over 32 days from May 01 to Jun 01

HR

Breathing

Low HR May 08 to May 08 (1d)	Low HR May 11 to May 14 (4d)	Low HR May 19 to May 22 (4d)	Low HR May 25 to May 25 (1d)
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9 episodic events Detected, spanning 12h total. Conditions: Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
9	12h	Low Heart Rate	<1	94%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 08	8 PM to 10 PM	2h	1	Low Heart Rate	43	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 11	3 AM to 4 AM	4h	3	Low Heart Rate	43	39	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 19	4 PM to 5 PM	5h	4	Low Heart Rate	44	41	51	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 25	12 AM to 1 AM	1h	1	Low Heart Rate	44	39	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

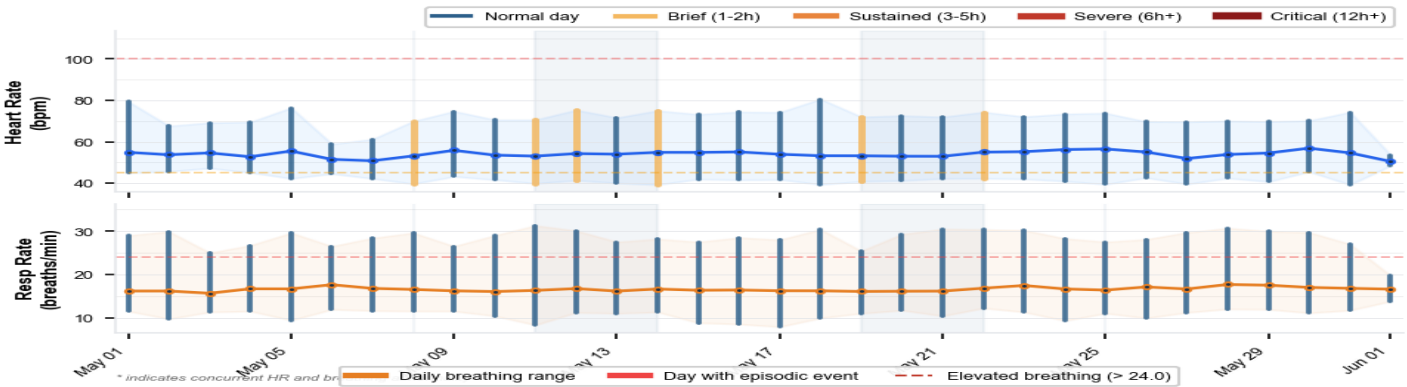
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 16 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

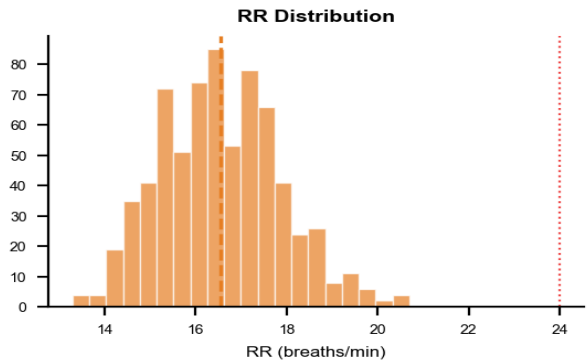
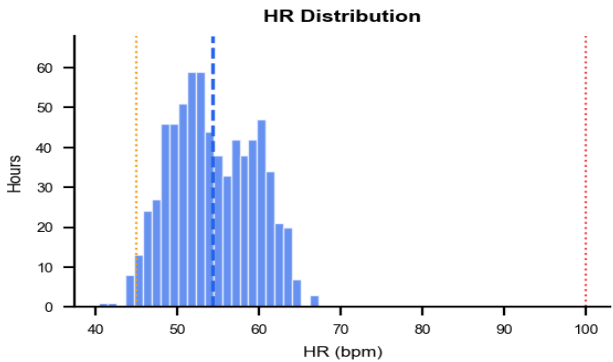
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 6 of 32 days (18%).
- 2. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.

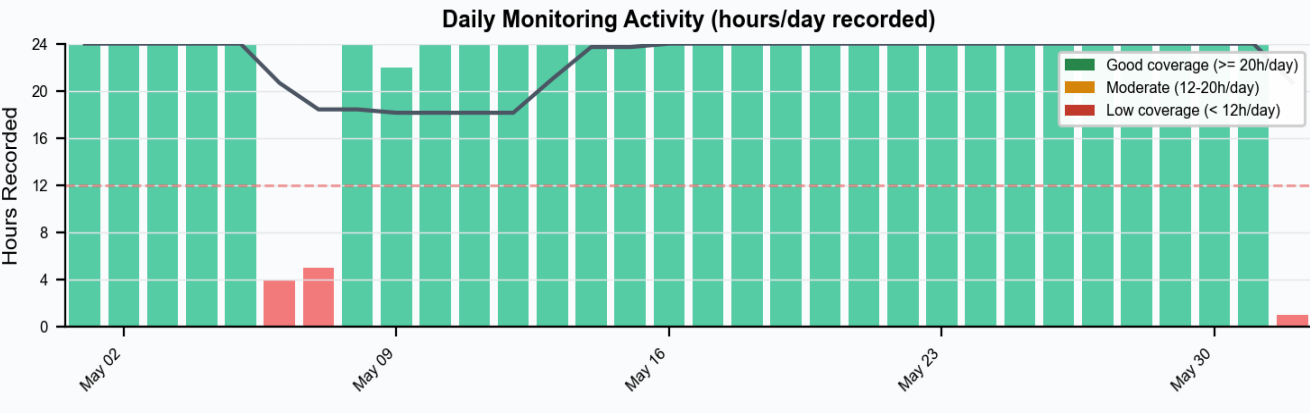


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode RR episode Recorded, no episode No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.